

Public Health Emergency Operations Center

"COUSP DRC"

MVE B-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°080/MVE B_02/08/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (51)	• Haut-Uélé (5/13): Boma Mangbetu, Isiro, Pawa, Rungu and Wamba • Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gethy, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. • North Kivu (12/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Musienene and Lubero. Masereka • , - South Kivu (1/34): Miti-Murhesa • Tshopo (5/23): Kabondo, Lubunga, Makiso-Kisangani, Mangobo and Wanie-Rukula
Reporting date	August 2, 2026
Publication date	August 3, 2026

INDICATORS KEYS — CURRENT TO JULY 31, 2026

CONFIRMED CASES — 5 PROVINCES 3,802 +58 new cases (Ituri 39 · N-Kivu 18 · Haut-Uélé 1)	CONFIRMED DEATHS · LEATHITIS 1707 · (44.9%) +40 deaths today (Ituri 23 · N-Kivu 14 · H-Uélé 3) · +10 per harmonization)	SUSPECTED CASES OF THE DAY 275 Ituri 172 · N-Kivu 102 · Haut-Uélé ND · South Kivu 1
PATIENTS IN ISOLATION 707 Ituri 468 · N-Kivu 186 · Haut-Uélé 31 · Tshopo 2. Sud Kivu 20	CURED — CUMULATIVE 727 +19 recovered (Ituri 17 · North Kivu 2)	NATIONAL CONTACT TRACKING 79.2% 14,466 / 18,276 views · Ituri 82.1% · N-Kivu 74.7% · H-Uélé 66.7% · Tshopo ND · Sud-Kivu 100%

1. Epidemiological Highlights

- **Epidemiological situation:** A new health zone has been affected in the last 24 hours: the Lubero zone in North Kivu, which has recorded its first confirmed case—a death. The province thus goes from 11 to 12 affected zones out of 34, and the country from 50 to 51 affected zones out of 140, across five provinces. North Kivu now has 73 affected health zones out of 684.
- **Case trends:** In the last 24 hours, 58 new confirmed cases, 37 confirmed deaths and 19 Recoveries were recorded; **+10 deaths by harmonization at NK.**
 - o **Ituri:** 39 new confirmed cases were reported in Rwampara (11), Bunia (9), Nizi (5), Bambu (4), Lita (4), Mongbwalu (2), Tchomia (2), Aungba (1) and Nia-Nia (1), as well as 23 deaths, including 14 community deaths (60.9%). (14) positive retests were also returned, bringing the total to 53 for the day.
 - o **North Kivu:** 18 new confirmed cases in Katwa (10), Beni (3), Musienene (2), Kalunguta (1), Vuhovi (1) and Lubero (1), including 9 deaths. After harmonization with the provincial table, the cumulative total reaches 414 confirmed cases and 285 deaths, representing a case fatality rate of 68.8% — the highest in the country — including 209 community deaths.

1. North Kivu Harmonization — The provincial cumulative figures used in this SitRep (414 confirmed cases and 285 deaths) and the distribution of confirmed cases by health zone presented the provincial situation as of August 2, 2026. The discrepancy with a simple report ($400 + 18 = 418$ cases; $262 + 9 = 271$ deaths) is due to retrospective reclassifications carried out by the province, primarily in Katwa (+6 cases, +14 deaths) and Butembo (-1 case, +2 deaths). The cumulative number of deaths thus increased by 23, while the province recorded only 14 confirmed deaths in the last 24 hours — 9 among the new confirmed cases and 5 at the CTE/CT; the remaining 9 are previous deaths retrospectively attributed.

- o **Haut-Uélé** : 1 new confirmed case, a positive swab returned for Isiro, and a retrospective harmonization which brings the cumulative number of deaths from 30 to 34. The province has a total of 61 confirmed cases and 34 deaths, representing a case fatality rate of 55.7%, spread across 5 out of 13 health zones. One escaped case has been recovered in Pawa.

- o **Tshopo** : No new confirmed cases or deaths have been reported. The provincial total remains at 7 cases confirmed, 5 deaths and 1 recovered, representing a case fatality rate of 71.4%.

South Kivu : The provincial total remains at 3 confirmed cases, including 1 death and 2 recoveries. Contact tracing is ongoing.

The report continues: 78 contacts listed, 23 discharged, and 55 under follow-up, all seen (100%), including 41 in cantonment; the note states, **"These are contacts of a confirmed death imported from Tshopo province ."**

The number of days without a new confirmed case in Miti-Murhesa is 53 days since June 10th, the actual interval is 52 days.

- **Analysis:** Ituri remains the epicenter with 87.2% of cumulative cases and 67.2% of daily confirmations, but the fact

The day's most significant development shifted to North Kivu: the province accounted for 31.0% of new cases, surpassed 400 confirmed cases, reached a twelfth health zone, and recorded a case fatality rate of 68.8%, compared to 41.7% in Ituri. This 27-point difference reflects very late detection—49 of the 102 suspected cases reported that day were community deaths, representing 48%—and an in-hospital mortality rate of 35%. Contact tracing declined to 79.2%, down from 80.1% the previous day, still far from the operational threshold of 95%.

The deterioration stemmed from North Kivu (74.7%) and Ituri (82.1%), where three zones—

Mangala (43.7%), Fataki (51.8%) and Bunia (61.8%).

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it

The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 51 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to fuel transmission.

Multisectoral coordination remains mobilized to

strengthen surveillance, intensify response interventions and limit the spread of the epidemic.

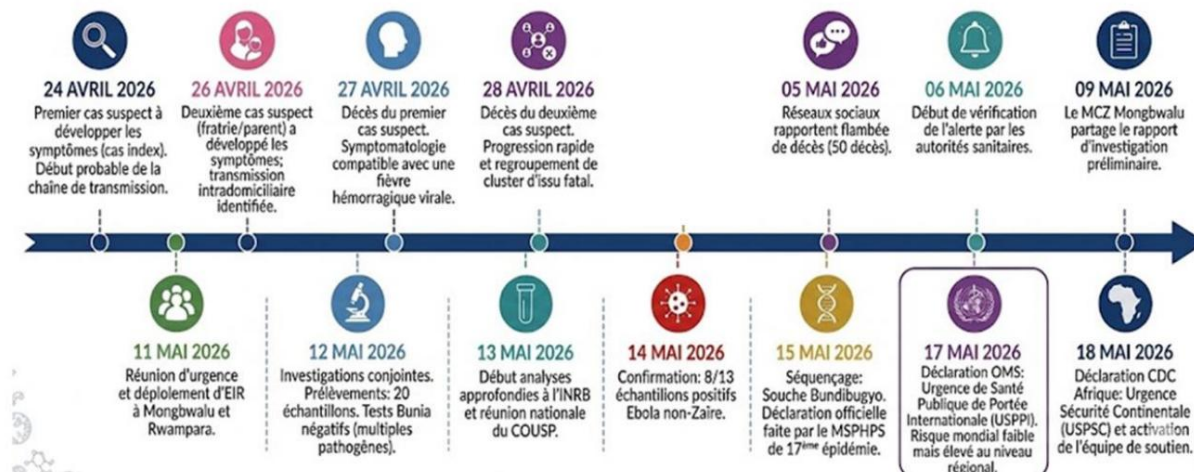


Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	3,317	1,382	41.7%	28 / 36 (77.8%)	39
North Kivu	414	285	68.8%	12 / 34 (35.3%)	18
Haut-Uélé	61	34	55.7%	5/13 (38.5%)	1
Tshopo	7	5	71.4%	5/23 (21.7%)	ND
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	3,802	1,707	44.9%	51 out of 140	58

Case fatalities recalculated from daily totals. The total number of affected areas includes Wanie-Rukula (Tshopo), confirmed on July 29, and Lubero (North Kivu), confirmed on August 2.

Ituri province accounts for 87.2% of confirmed cases (n = 3,317) and 81.0% of deaths (n = 1,382), with a case fatality rate of 41.7%. The health zones of Bunia (914 cases), Rwampara (655), Mongbwalu (552), Nizi (402), Lita (141), and Nyankunde (115) represent 83.8% of the cases reported at the provincial level and 73.1% of the national total. North Kivu, the second-largest outbreak hotspot, saw a 3.5% increase in cases in one day and its case fatality rate remains 27 points higher than that of Ituri.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Case (n)			Deaths (n)			
	3,317	1,382	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri							
Adja	11	1	41.5%	49	13	10	23
Aru	5	5	9.1%	0	0	0	0
Ariwara	7	2	100.0%	0	0	0	0
Aungba	10		28.6%	0	0	0	0
Bamboo	64	5 15	55.6%	0	0	0	0
Boga	1	1	21.7%	0	0		1
Bunia	914	270	100.0%	0	0	1	0
Damascus	18	8	29.0%	9	2	0 1	3
Drodro	11	6	44.4%		0	0	0
Fataki	35	17	54.5%		0	0	0
Gethy	1	0	48.6%	1	0	0	0
Kambala	2	0	0.0%	1 0 0	0	0	0
Kilo	29	11	0.0%	0	0	0	0
Komanda	37	27	37.9%	0	0	0	0
Lita	141	84	73.0%				2
Logo		4	58.4%	1	1		1
Lolwa	9 7	2	33.3%	5 0	0 0	1 1 0	0
Mahagi	4	1	28.6%	0	0	0	0
Mambasa	10	6	25.0%	0	0	0	0
Mandima		12	60.0%		0	0	0
Mangala		66	75.0%	1 0	0	0	0
Mongbwalu	16,105,552	263	62.9%	3	2	0	2
Nia-Nia	104	57	47.5%	5	2	0	2
Nizi	402	194	54.4%	2	1	2	3
Nyankunde	115	34	48.4%	10	3	2	5

Rimba		4	29.6%	0	0		2
Rwampara	9,655	263	44.4%	0	0		0
Tchomia	43	24	40.4%	11	2	200	2
North Kivu	414	285	58.5%	0	0	0	0
Blessed	64	49	65.5 %	22	7	4	11
Butembo	87	69	80.3%	4		2	3
Goma	1	0	76.1%	5	11	1	2
Kalunguta	7	4	0.0%	0	0	0	0
Katwa	186	122	50.0%	0	0	0	0
Kyondo	13	10	60.0%	12	5		6
Lubero	1	1	61.5%	0	0	10	0
Mabalako	1	0	0.0%	0	0	0	0
Masereka		1	0.0%	0	0	0	0
Musienene	543	25	56.1%	1	0	0	0
Oicha	4	3	75.0%	0	0	0	0
Vuhovi	2	1	100.0%	0	0	0	0
Haut-Uélé	61	34	33.3%	0	0	0	0
Boma	13	4	33.3%	0	0	0	0
Mangbetu							
Isiro	19	11	50.0%	3	0	2	2
Pawa	12	10	33.3%	0	0	0	0
Rungu		0	47.4%	0	0	0	0
Wamba	1 16	9	75.0%	2	0	2	2
Tshopo	7	5	0.0%	0	0	0	0
Kabondo	1	1	50.0%	1	0	0	0
Lubunga	1	0	71.4%	0	0	0	0
Makiso-	3	3	100.0%	0	0	0	0
Kisangani							
Mangobo			0.0%	0	0	0	0
Wanie-Rukula	1 1	10	100.0%	0	0	0	0
South Kivu	3	1	100.0%	0	0	0	0
Miti-Murhesa	3	1	0.0%	0	0	0	0
Total	3,802	1,707	44.2%	58	40	32	72

2.3 Mapping of confirmed cases

The spatial distribution confirms a transmission mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalu, Nizi, Lita and Nyankunde, with transmission continuing in the areas already affected without any new geographical expansion in the last 24 hours.

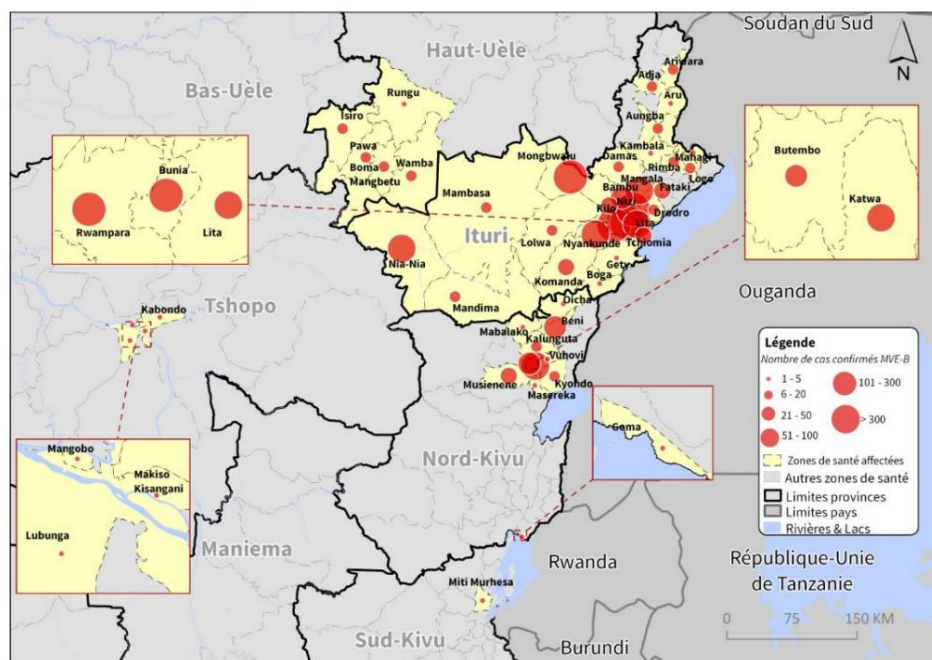


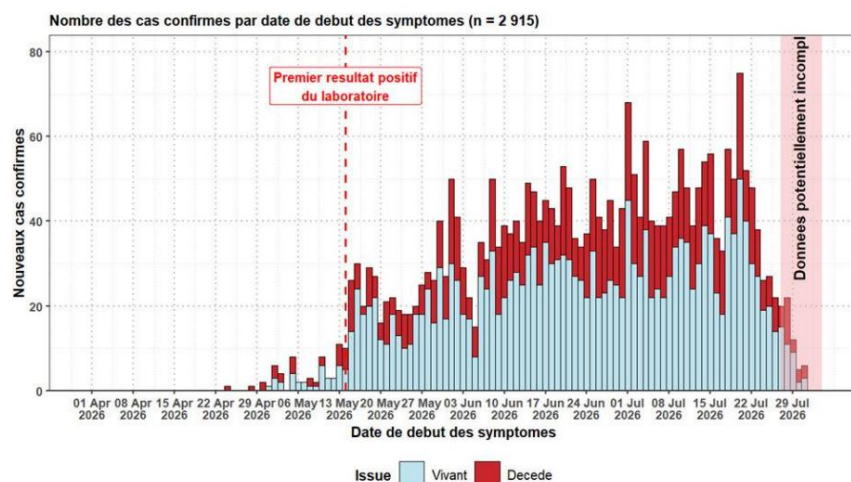
FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

Transmission in already affected health zones underscores the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S26 to S31, with fluctuations related to notification delays and the gradual consolidation of data.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION



Source : DHIS2 Tracker - Riposte MVE, août 2026. Données incluses : cas confirmés avec statut vital et date de début de symptômes renseignés, n = 2 915.

FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults (18–50 years)**. Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

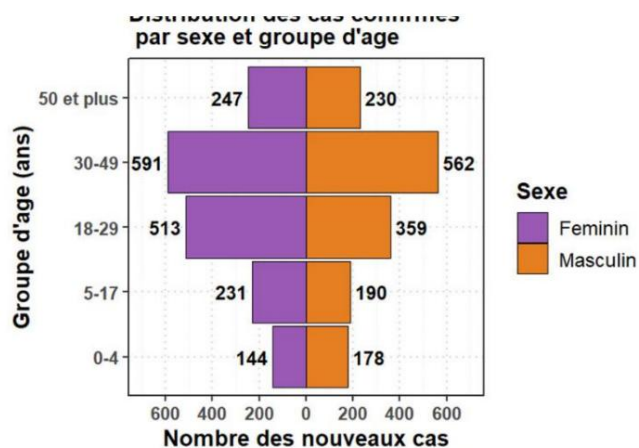
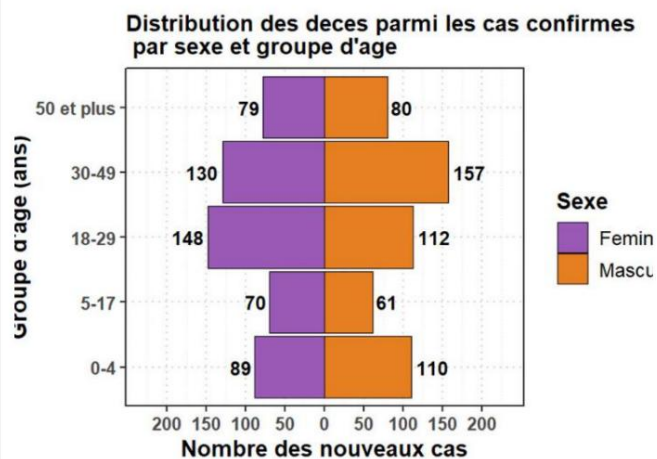


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	15	0	ND	ND	15
New alerts received	507	731	ND	ND	1,241
Total alerts for the day	531	731	ND	ND	1,265
Alerts investigated	433	728	ND	ND	1,164
Investigation rate (24 h)	81.5%	99.6%	ND	ND	92.0%
Alerts confirmed — live	98	53	ND	ND	152
Alerts confirmed — deceased (comm.)	74	49	ND	ND	123
Total suspected cases for today	172	102	ND	ND	275

In total, 1,265 alerts were managed (including 1,241 new ones), 1,164 were investigated within 24 hours (92.0%), and 275 were validated (152 living cases and 123 community deaths). North Kivu now accounts for the majority of the volume with 731 alerts investigated at 99.6%, compared to 531 alerts in Ituri investigated at 81.5%. This reversal is noteworthy: North Kivu more than doubled its number of alerts in a single day (571 the previous day) while maintaining its investigation rate, but its reporting completeness remains limited to 25 out of 34 zones (73.5%). South Kivu is investigating all 3 of its alerts. Haut-Uélé still has not submitted any alert management slides, and Tshopo has not provided any decks: their indicators are marked ND, and the national totals only cover three provinces.

3.2 Laboratory: tests and positivity

Ÿ **Ituri** : 210 samples documented in the collection network, including 53 positive (25 live and 14 dead), — 39 new cases confirmed and 18 re-samples, i.e. an apparent positivity of 25.2%.

Ÿ **North Kivu**: 132 samples received, of which 47 were analyzed in Beni (3 positive) and 85 in Butembo (15 positive), resulting in 18 positive cases and one negative result. positivity rate of 13.6%; 5 analyses remain in progress.

Ÿ **Haut-Uélé**: 4 samples received and analyzed, 1 positive (Isiro swab), no invalid results, 1 re-sample. Only one of the 13 planned laboratories is operational; six RadiOne machines have been received from the WHO and deployment is planned in Wamba, Boma Mangbetu and Rungu.

Ÿ **Tshopo**: no samples were reported for August 1st; the provincial total stands at 7 positive cases out of 51 tests. Four samples had been taken from 20 alerts on July 30.

Ÿ **South Kivu**: 1 sample received (Bagira), negative. The provincial total stands at 3 positives out of 296 samples analyzed (1.0 %); 94% of samples are analyzed within the day following collection.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	11,496	9,434	82.1%
North Kivu*	6,163	4,602	74.7%
Haut-Uélé**	562	375	66.7%
South Kivu	55	55	100.0%
Tshopo***	ND	ND	ND
Total	18,276	14,466	79.2%

* *Ituri: The provincial table shows a total of 531 alerts, while the carryover (15) and new alerts (507) total only 522; the discrepancy of 9 comes from three zonal lines whose internal sum does not complete (Bambu, Mangala, Mongbwalu). The investigation rate is recalculated at 81.5%, compared to the 84% announced by the province.*

Contact tracing has fallen to 79.2% nationally, compared to 80.1% the previous day, and remains well below the operational threshold of 95%. Ituri is monitoring 9,434 out of 11,496 contacts (82.1%) and has seen significant progress in volume—510 new contacts listed—but three areas account for 76.5% of the 2,062 untraceable contacts: Mangala (904 untraceable, 43.7% follow-up), Fataki (122, 51.8%), and Bunia (552, 61.8%). The areas of Kambala, Logo, and Damas remain at zero listed contacts despite having cumulative cases.

North Kivu reports 4,602 contacts seen out of 6,163 (74.7%), with 152 lost to follow-up and 915 never seen; Beni (69%), Butembo (69%), and Katwa (67%) are dragging down the provincial performance. South Kivu has seen all 55 of its contacts currently being followed up, with 45 contacts in cantonment.

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 4 alerts notified

Four have been notified at the province's border crossings in the last 24 hours.

North Kivu — alerts notified

No alerts were notified to the province's activated PoE/PoC devices.

Tshopo — not reported

No alerts were notified to the province's activated PoE/PoC devices.

Haut-Uélé — 0 alerts notified

No alerts were notified to the province's activated PoE/PoC devices.

3.4.2 Monitoring indicators for PoE/PoC**TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)**

Indicators	Ituri North Kivu		South Kivu	Tshopo	High-Uele	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (18/18)	100% (9/9)	21.1% (4/19)	30.3% (10/33)	72.7%
Number of people who have switched to PoE/PoC	98,289	65,338	89,416	ND	ND	253 043
% of travelers screened	96.5%	97.3%	100.0%	ND	ND	98.0%
% of travelers who washed their hands	96.7%	97.3%	100.0%	ND	ND	97.6%
% of travelers made aware	99.1%	97.3%	100.0%	ND	ND	98.9%
Number of alerts notified	4	0	0	ND	ND	4
Number of validated live alerts	2	0	0	ND	ND	2
Lifeless bodies intercepted today	1	0	0	ND	ND	1
Problem contacts intercepted today	0	0	0	ND	ND	0
% of validated live alerts referred to CT/CTE	0% (0/2)	0	0	ND	ND	0%
% of lifeless bodies swabbed	100% (1/1)	0	0	ND	ND	100% (1/1)
Positive results from referred suspected cases	0	0	0	ND	ND	0

4. Support**TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI)**

Indicator	Ituri	North Kivu	Haut-Uélé	Tshopo	South Kivu
Number of beds	821	141	68	20	ND
Patients in bed (Day -1)	476	183	33	2	17
Total admissions (24 hours)	70	39	16	ND	22
Total admissions (cumulative)	975	451	79	3	287
Exits — deceased	13	5	4	ND	0
Exits — non-cases	46	28	0	ND	19
Exits — cured	17	2	0	ND	0
Exits — escapees	2	0	0	ND	0
Discharges — transferred to the HGR	2	0	0	ND	0
	0	0	0	ND	0
Total outflows (24 h)	78	35	4	ND	19
Patients in isolation (end of day)	468	186	31	2	20
including those confirmed	296	25	13	1	0
including suspects	172	161	18	1	20
Occupancy rate	57.0%	131.9%	45.6%	10.0%	ND

In total, 707 patients were in isolation, including 335 confirmed cases and 372 suspected cases, with 147 admissions and 136 discharges (19 recovered, 22 deaths, 93 non-cases, 2 escaped). Ituri accounts for the majority of the burden with 468 patients in isolation (296 confirmed, 172 suspected) out of 821 beds, representing 57.0% occupancy and a daily change that exactly matches the previous day's total ($476 + 70 - 78 = 468$); its However, the indicator slide continues to reverse the numbers of confirmed and suspected cases, with the breakdown by site being the determining factor. Sixty-three patients are in serious or critical condition. Six treatment centers are over capacity for confirmed cases—Nizi at 278%, ISTM at 123%, CME at 104%, Lita and Fataki at 100%—and two for suspected cases, Fataki at 200% and Aru at 140%. In North Kivu, 186 patients are in isolation out of 141 beds, representing an occupancy rate of 131.9%: this is the most acute point of strain on the national system, exacerbated by a bed capacity that has not been reported for seven cycles. Haut-Uélé has 31 patients out of 68 beds (45.6%), but its daily patient flow is still not complete—45 patients expected versus 31 reported—and its patient care charts are identical to those of the previous day. South Kivu, which had not been included in the figures until now, has isolated 20 suspected cases distributed between Lwiro, the University Clinics of Bukavu and Katana

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that have received interventions	6/15 (40.0%)	3/3 (100%)
Confirmed cases are being monitored.	132 / 188 (70.2%)	7/7 (100%)
New suspected cases	16/32 (50.0%)	9/9 (100%)
Suspected cases are being monitored	113/117 (96.6%)	10/10 (100%)
Laboratory results announced (including positive ones)	70 (7)	3 (3)
Children separated — nursery / community	0 / 25	0 / 0
CTE support staff	164	10
Frontline Personnel (PPL)	22	0
Members of households and communities	48	0
EDS supported (SMSPS)	8	ND

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD IN ITURI

PPL = person

Health Zone	Confirmed cases	Healed	In hospital	Death
Bunia	35	19	1	14
Rwampara	33	25	1	7
Nia-Nia	22	5	12	5
Mongbwalu	17	7	1	9
Nyankunde	14	11	1	2
Lita	4	1	2	1
Mangala	4	2	1	1
Nizi	3	0	0	3
Logo	3	0	3	0
Bamboo	2	2	0	0
Kilo	2	1	1	0

Health Zone	Confirmed cases	Healed	In hospital	Death
Damascus	1	0	0	1
Total — case fatality rate 30.7%	140	71	23	43

healthcare professionals and frontline providers.

In Ituri, 140 cases of PPL (Patients of Locally Affected Persons) have been confirmed, including 43 deaths, representing a case fatality rate of 30.7%. This total is down by two cases compared to SitRep No. 079 (142), due to reclassifications in Mongbwalu (21 ÷ 17) and Nizi (4 ÷ 3), partially offset by Nia-Nia (20 ÷ 22) and the appearance of a case in Damas. A decreasing total violates the rule against regression and requires arbitration by the provincial division. Furthermore, the breakdown by outcome shows only 139 PPL cases—the sum of recoveries per zone is 73, while the total reported is 71. The IPC (Infection Prevention and Control) pillar shows a cumulative total of 134 infected PPL cases out of 2,884 exposed (4.6%), unchanged for three editions, a persistent discrepancy that needs to be reconciled with the surveillance register. In North Kivu, 11 cases of PPL are confirmed, including 3 deaths; Haut-Uélé reports 3 cases, representing 4.9% of its confirmed cases, including one hospitalized at the Isiro CTE.

7 Pillar-Based Response Activities

COORDINATION ÿ Organization of the workshop for updating the response plan in the context of the 5 affected provinces.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ÿ Ituri: Supervision of ESS (Emergency Health Services). Ituri: training of community health workers and supervision in Mongbwalu, training of IT (Information Technology) and PRESICOSA (Health Promotion and Support Center); 34 ambulances, 30 of which are available, 28 trips completed and 30 patients evacuated, 3 ambulances out of service and a reporting completion rate of 50%. Receipt of a Land Cruiser vehicle from the government via the UG-PDSS (Departmental Unit for Health and Social Services). ÿ North Kivu: continued analysis of supporting documents for ECZS and preparation of July payroll statements.
LOGISTICS ÿ Ituri: Ensuring the mobility of different pillars of the response. ÿ North Kivu: ensuring the reception and delivery of supplies PCI	PCI / WASH ÿ Ituri: 53 rings opened and 53 decontaminated, 27 ESS monitored and supported, 5 ESS that had housed a confirmed case decontaminated within 48 hours (100%); 53 households out of 136 decontaminated within the time frame (38.9%) and 26 providers trained in Komanda). ÿ North Kivu: decontamination of a household of confirmed case and of an ESS staying in Musienene, provision of incomplete PCI kits to two ESSs, monitoring of 66 ESSs and briefing of 228 providers; completeness limited to 6 out of 11 zones.
COMMUNICATION (RCCE / CREC) ÿ Ituri: 5,158 people reached at home, 2,331 during 25 talks, 2,129 through mass awareness campaigns; 102 community alerts reported (89 living, 13 deceased). Of 19 feedback points, 7 were clarified (37%), and the single instance of resistance recorded in Nyankunde remains. ÿ North Kivu: 17,648 households visited out of 25,500 (69.2%) and 38,596 people reached through door-to-door canvassing; public forums in Nduko (Musienene) and Butsili	HOLISTIC CARE (HCC) ÿ Ituri: 17 new recoveries; 468 patients in isolation (296 confirmed, 172 suspected) and 63 severe or critical cases. Six Ebola Treatment Centers (ETCs) are over 100% occupancy for confirmed cases — Nizi (278%), ISTM (123%), CME (104%), Lita and Fataki (100%) — and two for suspected cases (Aru 140%, Fataki 200%). Nutritional support: 1,030 hot meals served. ÿ North Kivu: 39 admissions and 35 discharges. Regarding nutrition, 2 confirmed unstable patients are on special diets.

(Beni); 278 alerts of suspects reported out of 450 expected (61.7%) and all 34 targeted radio stations broadcast. ÿ South Kivu: 1,100 community actors and 120 Religious leaders trained, 12,694 home visits for 140,712 people reached, 1,951 community alerts raised; 12 rumors collected and all resolved within 72 hours. ÿ Haut-Uélé : 6,414 people received mass awareness training, including support from the vice-governor in Wamba, and 1,271 reached at home; an exploratory visit to the Pygmy camp of Nazareth (Telly, Isiro) led to a community death alert.	specific, 58 confirmed and 41 suspected cases being fed, 3 infants receiving breast milk substitute... ÿ Haut-Uélé: 16 admissions and 4 deaths out of 31 patients at the end of the day; service has begun at the Isiro CTE and renovation work is underway. The Wamba CTE is still exceeding its capacity for confirmed cases (125%).
RESEARCH, INNOVATION & VACCINATION ÿ Ituri: ND	Dignified and secure burials (DSM) ÿ Ituri: 99 death alerts, 81 EDS carried out (82%) and 60 swabs, including 26 in CTE; 16 out of 32 zones reported. Nine refusals persist — Nia-Nia (5), Mandima (2), Bunia (1), Nyankunde (1). The formation of two teams in Tchomia has concluded, and three ECUMER teams have begun training in Lita. ÿ North Kivu: 42 death alerts, 42 bodies recovered and secured, 40 mass casualty incidents (MSIs) carried out with no failures; 13 bodies remain pending. Deaths occurring in the ESS (Emergency Situations) predominate, in Beni (17) and Katwa (17). ÿ Haut-Uélé: EDS activities integrated into the PCI/WASH pillar. ÿ South Kivu : 22 suspects admitted, 19 discharged as non-cases, no deaths; 20 suspects hospitalized in Lwiro, CUB and Katana.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) ÿ Ituri: continuation of briefing and awareness-raising by the focal points of the Fataki and Mandima health zones, and continuation of the cascade briefing in the Ariwara health zone with the support of the WHO.	

8 Main challenges, impacts and actions required

Challenge No.		Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of the military governor of Ituri)	Interruption of response activities, including dignified and safe burials	Regularize payments to service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT facilities: 128.4% occupancy in North Kivu (capacity of 141 beds: not reported for six cycles) and five facilities beyond or at the limit of their capacity for confirmed cases in Ituri, including the Nizi CTE at 278% and ISTM CTE at 110%;	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report bed capacity.
4	Insufficient contact tracing performance: Ituri 83.4%, North Kivu 75.0% and South Kivu 100%, the Haut-Uélé and Tshopo did not provide data; the health zones of Damas, Logo and Kambala (Ituri) remained without usable data	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs); strengthen supervision; consolidate the recovery observed in Ituri
5	Insufficient number of ambulances and pre-positioning at PoC	Delays in transfer and isolation	Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (181/1170, i.e. 15%)	Risk of nosocomial infections (141 cases of PPL, including 41 deaths in Ituri; 11 PPL, including 3 deaths in North Kivu and 3 PPL confirmed at Haut-Uélé)	Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of local laboratories (Rwampara, Fataki, Bambu-mine, Boga and Mambasa)	Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
10	Inadequacy of the financial resources	Limitation of the implementation of pillars	Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations	Strengthening security coordination and humanitarian access

Challenge No.	Impact	Actions required
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality
		Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities

Awareness-raising among Christians from CCA 20 in the N'Sele district: 620 people participated



Receipt of the RADIONE(6) machines with the support of the WHO



FOR ANY INFORMATION

**ADDITIONAL INFORMATION, PLEASE
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